Inpatient care

Inpatient care is care rendered to a person who has been admitted to a hospital for bed occupancy for purposes of receiving inpatient hospital services. Generally, a patient is considered an inpatient if formally admitted as an inpatient with the expectation that he or she will remain at least overnight and occupy a bed even if it later develops that the patient can be safely discharged or transferred to another hospital and not actually use a hospital bed overnight. See Section 3, *How You Get Care*, Covered facilities, for the definition of an Acute Inpatient and Residential Treatment Center.

Long-term acute care

Often referred to as LTCH or LTAC, these facilities provide services for patients who need extended intensive or critical, hospital-level of care beyond that of the traditional short hospital stay. LTCH's specialize in treating patients who have more than one serious condition yet have the potential to improve with time and care and return to their previous health status. Generally, services are focused on respiratory therapy, head trauma treatment, and pain management.

Long-term care

We do not provide benefits for long-term care, regardless of who recommends the care or where it is provided. The Carrier or its delegated medical professionals determine which services are long-term care.

A range of services and support provided to meet personal care needs on a long-term basis. While some medical care may be necessary, most of the care provided is not and does not require a licensed caregiver. This encompasses a spectrum of services provided in a variety of settings for people who do not have full independence because of a medical condition, injury, or chronic and/or behavioral illness.

Long-term care is often used to provide custodial care as well as instrumental activities of daily living necessary for safety and health.

Long-term care is usually custodial care that has lasted for 90 days or more yet can begin prior to 90 days dependent on the member's response to professional intervention.

Long-term care and long-term acute care are not one and the same. See the definition of long-term acute care for more information about those services.

Medical emergency

A medical emergency is the sudden and unexpected onset of a condition or an injury that you believe endangers your life or could result in serious injury or disability and requires immediate medical, surgical, or behavioral health care (includes mental health and substance use disorders).

Medical foods for inborn errors of metabolism (IEM)

Inborn errors of metabolism are rare genetic (inherited) disorders in which the body cannot properly turn food into energy. The disorders are usually caused by defects in specific proteins (enzymes) that help break down (metabolize) parts of food. G.E.H.A will cover medical food for a diagnosis of IEM. Medical Food is defined as a food which is recommended by a physician after an evaluation and is intended to provide for the dietary management of a disease or condition that has specific nutritional requirements. G.E.H.A will not cover "grocery" food items that can routinely be obtained online or in stores (e.g., gluten-free breads).

Medical necessity

Healthcare services, drugs, supplies or equipment provided by a hospital or covered provider of the healthcare services for the purpose of preventing, evaluating, diagnosing, or treating a Medical or Behavioral Health Condition, that the Plan determines are all of the following:

- Are clinically appropriate to diagnose or treat the patient’s condition, illness or injury in terms of type, frequency, extent, site and duration of treatment, and treatment setting are considered effective for your condition.
- Are consistent with generally accepted standards of medical practice in the United States.
  - Generally accepted standards of medical practice are based on credible scientific evidence published in peer-reviewed medical literature generally recognized by the relevant medical community, nationally recognized evidence-based clinical guidelines and criteria from independent sources, national physician specialty society recommendations and the views of medical practitioners practicing in relevant clinical areas, and any other relevant factors. Please refer to geha.com/Coverage-Criteria for details on the literature used in medical decision making.